

Stream — Field One-Pagers

Internal Medicine & Risk-Bearing Primary Care 19 Aug 2026 For the GTM team — not for distribution to prospects

FIELD SHEET

Internal Medicine & Risk-Bearing Primary Care

Heaviest fax volume of the three, and the only segment where the buyer already has a dollar figure in their head. Qualify hard on risk exposure.

WHO TO CALL, WHO TO SKIP

CALL

- + Independent IM / adult primary care, **3–20 clinicians** (four or more preferred)
- + Risk exposure: Medicare Advantage, ACO participation, any capitated contract
- + Buyer is the owner, managing partner, or whoever answers for risk and quality

SKIP

- Health-system owned
- **Under 3 clinicians** — self-serve trial, not a booked demo
- Pure fee-for-service with no fax pain — that's a scribe buyer who will grind you on price

THE OPENER

Which of your Medicare Advantage patients haven't had their conditions documented this year — and how do you find out?

The answer is a spreadsheet, a vendor report that arrives two months late, or nobody. If they're not risk-bearing, fall back to: *"Who reads the hospital discharge summaries?"*

THE THREE BEATS

- 1 Walk in ready.** Four tabs before you open the door: what's active, what's falling off the chart, what's HCC-coded and not yet documented this year, and what's waiting on your review.
- 2 An eight-problem patient gets an eight-problem note.** Each condition documented separately with its own assessment and plan — not one paragraph somebody has to unpick later. Codes suggested, then sharpened to the more specific one.
- 3 Nothing falls off.** Discharge summaries and consult letters file under the right problem. A condition that shows up *only* in the outside record becomes a real problem on the chart, flagged external — the hospital found something your chart didn't know about.

THE DEMO

Discharge summary first, same as every segment. Then open the pre-visit view and show **Falling Off** — problems

last documented ten or eleven months ago, red badge at eleven. Say: *"That's your December scramble, in August."*

THE NUMBER — MAKE THEM BUILD IT

Ask what their average RAF is and what a recaptured condition is worth under their contract. Stream carries HCC weights to four decimal places and shows them on the problem itself. Their finance person already owns the multiplier; you just supply the conditions they're about to lose.

Honest ramp.

The HCC surfaces work from what's documented in Stream, so they sharpen over the first months as patients cycle through — faster here than pediatrics, because these patients come in three or four times a year. The document workflow works week one. Say both.

OBJECTIONS

"Our ACO already sends us gap reports."

Those are retrospective and claims-based — they tell you what you missed. This tells the clinician what's open while the patient is still in the room. Different job, different month.

"That's double documentation."

It is a paste, not a retype — one click, formatted for any field, section by section if that's how your EHR wants it. Acknowledge and move.

"Does it hallucinate?"

Every outside claim links to the highlighted sentence it came from; anything unverifiable is suppressed; outside content never enters a note without the clinician putting it there. Demo it.

"What about compliance?"

BAA included in our terms — no separate negotiation. Per-tenant database isolation, role-based access control, audit logging, two-factor authentication. **Do not say SOC 2.**

PRICING

Pro \$149/clinician/month, or \$99 billed annually · staff seats **\$25** (first free) · 30-day trial, no card · USD and CAD. Document upload included; provisioned fax number free for the first 30 days.

APPLIES TO EVERY SEGMENT

Guardrails

Everything here was verified against the product, not the roadmap. A claim from the left column is the fastest way to lose a deal in diligence.

Booking floor — 3 clinicians minimum, 4 or more preferred.

No demo with a practice under three clinicians. At \$149 a seat, a one- or two-person practice doesn't cover the cost of sourcing the meeting — that demand is real, but it arrives through the self-serve trial, not a booked call.

Confirm headcount before the invite goes out. Count physicians, NPs and PAs; not front-desk, billing or MAs — those are \$25 seats.

NEVER SAY / SAY INSTEAD

NEVER SAY	SAY INSTEAD
✗ Integrates with your EHR · syncs with · connects to · writes back	Works alongside any EHR. No integration, no IT project, no implementation fee. <i>(The old "native integrations coming" claim was removed from the website in Aug 2026 — it was never true. Don't reintroduce it on a call.)</i>
✗ SOC 2 compliant / certified	BAA included in our terms. Per-tenant data isolation, role-based access control, audit logging, two-factor authentication.
✗ HIPAA certified	HIPAA-aligned architecture. (No vendor can be HIPAA certified — the phrase reads as amateur to anyone who'd know.)
✗ Your overdue list is ready on day one	The document workflow works week one. The panel view fills in as your patients come through.
✗ Any mention of Form Filler, automatic chart extraction into structured tables, or style-learning from a sample note	Nothing. Not shipped. Don't hint at it either.
✗ Unlimited history on every plan	Lite / Quick Encounter deletes after 48 hours — state it plainly in all Lite material or it's a refund and a support fire. Lite (\$59, scribe only, no chart) isn't publicly sold: never put a practice on it.
✗ Clinicians save X minutes a day	We have no time-saved statistic — don't invent one. We <i>do</i> have three named physician references and 30,000+ documented encounters. Use those, and build the ROI number out of the prospect's own figures.

BEFORE ANY DEMO

Confirm flag state on the account — these are per-tenant and may be off:

task_rules document_excerpts tasks_module

outbound_faxes journal_club help_chatbot

merge_inline_recommendation

Documents still carries a **Beta** badge. One-click demo charts exist for family medicine, pediatrics, cardiology, endocrinology and behavioral health — ask Jacob for access, and **demo a pediatrician a pediatric panel.**

SET THESE EXPECTATIONS IN ONBOARDING, NOT IN THE WILD

- Fax needs a Spruce account or a Stream-provisioned number: ~15 minutes of admin *plus* a manual step on our side. Not self-serve — don't promise same-day fax.

- Excerpt review is one-way: Accept and No-action are final (Re-tag is the exception).
- Regenerating a note replaces manual edits. Teach generate-once-then-edit-in-place — the most common way a new user loses work.

WHAT WE STILL OWE THE FIELD

- **Proof.** Three named physician references and 30,000+ documented encounters exist and are on the website — use them. Still missing: retention figures and a defensible time-saved number, so ROI is still built from the prospect's own numbers.
- **A security page.** Small-practice buyers ask, and the Trust Center link is currently commented out of the app.
- **Prior-well-visit import.** The single build that would turn the pediatrics sheet from a promise into a demo.

Derived from the engineering handoff of 19 Aug 2026 and a direct read of the source. Claims about what the product does were checked in code; claims about who wants it are hypotheses with one design partner behind them. Update this sheet when that changes.